Table: 3O

Parameters	Gotistobart N=45	Docetaxel N=42
Confirmed ORR by investigator, n (%) [95% CI]	9 (20.0) [9.6, 34.6]	2 (4.8) [0.6, 16.2]
Median DoR by investigator, m (95% CI)	11.0 (3.5, NE)	3.8 (3.5, NE)
Median PFS by investigator, m (95% CI)	2.4 (2.1, 4.5)	2.6 (2.1, 3.9)
PFS rate at 6 m, % (95% CI)	32.7 (19.1, 47.1)	12.9 (4.2, 26.7)
PFS rate at 12 m, % (95% CI)	25.2 (13.2, 39.2)	0
PFS HR (95% CI)	0.69 (0.42, 1.13)	
Median OS, m (95% CI)	NE (9.3, NE)	10.0 (6.2, 11.9)
OS rate at 12 m, % (95% CI)	63.1 (46.9, 75.5)	30.3 (16.2, 45.6)
OS HR (95% CI)	0.46 (0.25, 0.84)	

Conclusions: Gotistobart monotherapy, offering a chemotherapy-free option, demonstrated durable anti-tumor activity with clinically meaningful survival benefit compared to docetaxel in sqNSCLC after progression on anti-PD-(L)1 therapy, addressing a critical unmet need for effective treatments in this population. The pivotal Stage 2 part is enrolling patients with sqNSCLC globally.

Clinical trial identification: NCT05671510; EUCT 2023-505311-20-01.

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4MO Updated results for MK-1084 + pembrolizumab in KRAS G12C-mutated (mut) metastatic non-small cell lung cancer (mNSCLC) enrolled in KANDLELIT-001

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Background: The phase I KANDLELIT-001 study (NCT05067283) showed manageable safety and promising antitumor activity for the oral, next-generation, selective KRAS G12C-GDP covalent inhibitor MK-1084 given as monotherapy or as part of combination therapy in participants (pts) with KRAS G12C-mut solid tumors. We report updated data for pts with KRAS G12C-mut mNSCLC.

Methods: Pts in arms 1+3 had any KRAS G12C-mut solid tumor with ≥1 prior systemic therapy and received MK-1084 monotherapy 25-800 mg/d. Pts in arm 2 had untreated mNSCLC of PD-L1 TPS ≥1% and received MK-1084 25-400 mg/d + pembro 200 mg Q3W. Pts in arm 4 had untreated nonsquamous mNSCLC and received MK-1084 50-200 mg/d + pembro 200 mg, carboplatin, and pemetrexed Q3W. Dose-limiting toxicities (DLTs) and safety were primary endpoints; ORR, DCR, and PFS per RECIST v1.1 by investigator review were secondary.

Results: At data cutoff on July 23, 2025, there were 146 pts in arms 1+3, including 32 (22%) with mNSCLC, 87 pts in arm 2, and 46 pts in arm 4. Median study follow-up was 27.9 mo, 14.1 mo, 14.6 mo, and 12.1 mo, respectively. The ORR in arm 2 (MK-1084 + pembro) was 92% (TPS ≥50%), with corresponding median PFS of 29 mo (Table). 15 (10%) pts in arms 1+3, 26 (30%) pts in arm 2, and 28 (61%) pts in arm 4 had gr 3-4 drug-related AEs, including 3%, 8%, and 11%, respectively, with gr 3-4 ALT increased and 3%, 9%, and 9% with gr 3-4 AST increased. 1 pt died from drug-related myelosuppression and platelet count decreased (arm 2), and 1 from drug-related immune-mediated myositis (arm 4).

Table: 4MO

Arm	n[a]	ORR % (95% CI)	DCR % (95% CI)	Median PFS (95% CI), mo
1+3 NSCLC (MK-1084 alone)	29	34 (18-54)	76 (56-90)	7.0 (4.0-16.4)
2 (MK-1084 + pembro)	81	77 (66-85)	96 (90-99)	28.9 (15.2-NR)
PD-L1 TPS 1%-49%	33	55 (37-77)	94 (80-99)	25.1 (9.5-NR)
PD-L1 TPS ≥50%	48	92 (80-98)	98 (89-100)	28.9 (20.6-NR)
4 (MK-1084 + pembro + chemo)	46	59 (43-73)	89 (76-96)	NR (8.5-NR)

[a]Pts with ≥1 MK-1084 dose ≥7 wks before data cutoff.

Conclusions: MK-1084 + pembro in KANDLELIT-001 showed clinically meaningful activity, including >90% ORR in pts with PD-L1 TPS ≥50%. AEs were manageable, with rates of high-grade increased ALT/AST <10%. The favorable benefit-risk of MK-1084 + pembro strongly supports the ongoing phase III KANDLELIT-004 study of first-line MK-1084 + pembro vs placebo + pembro for KRAS G12C-mut mNSCLC with PD-L1 TPS ≥50%.

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5MO Phase II ETOP ADEPPT trial: Adagrasib in patients with KRASG12C-mutant NSCLC who are elderly or have poor performance status - Final results

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Background: $KRAS^{G12C}$ mutations occur in 10-15% of NSCLC. Adagrasib is a 2nd generation KRAS G12C inhibitor, that has demonstrated activity in pre-treated NSCLC patients (pts) with ECOG performance status (PS) 0—1. However, a large proportion of KRAS+ NSCLC pts are elderly or have an ECOG PS 2. This study evaluates efficacy, toxicity and quality of life (QoL) of adagrasib in these underrepresented populations.

Methods: ADEPPT (NCT05673187) is a prospective, international, phase II trial of adagrasib (600 mg bid until progression or unacceptable toxicity) in $KRAS^{G12C}$-mutant pre-treated NSCLC, with two parallel single-arm cohorts. Eligible pts were ≥70 years old with ECOG PS 0—1 (cohort A [A]) or ≥18 years old with ECOG PS 2 (cohort B [B]). The primary endpoint was confirmed objective response rate by 12 weeks (cORR) by RECIST v1.1. A sample size of 34 pts (per cohort) provided 80% power to reject the H_0 of ORR ≤15% vs 35%, with the success criterion of ≥10 responders by 12 weeks (1-sided alpha 2.5%, exact binomial test). Progression-free survival (PFS), overall survival (OS), safety and QoL (NCCN-Functional Assessment of Cancer Therapy Lung Symptom Index-17) were secondary endpoints.

Results: From 12/06/2023-18/02/2025, 32/34 pts were enrolled in cohorts A/B. In A/B: 66%/62% were male, 97%/97% former-current smokers, 100%/91% adenocarcinomas, 16%/32% had baseline brain lesions. Median (m) follow-up was 16.8/18.5 months (mo). Mean relative dose intensity was 72%/79%. In cohort A, the primary endpoint was met. The cORR was 31% (10 confirmed responses). Overall ORR was 34%, mPFS 7.6mo, mOS 9.5mo, mDoR NR. In cohort B, the primary endpoint was not met. The cORR was 18% (6 confirmed responses). Overall ORR was 38%, mPFS 2.7mo, mOS 4.3mo, mDoR 2.4mo. Grade ≥3 treatment-related adverse events (TRAEs) were experienced by 41%/62% of pts in A/B, with 1 fatal TRAE (sudden death NOS, cohort B). At 12 weeks, mean increase in the QoL index from baseline was 2.3/6.6 in A/B, based on 18/11 pts still on treatment.

Conclusions: Adagrasib demonstrated meaningful clinical benefit with a manageable safety profile in elderly pts with ECOG PS 0—1, while activity was limited but with improved QoL for pts with ECOG PS 2.

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M.R. Garcia Campelo: Financial Interests, Personal, Invited Speaker: Janssen, Takeda, Pfizer, Roche, AstraZeneca, Novartis, Lilly, AstraZeneca, Janssen; Financial Interests, Personal, Advisory Board: MSD, Janssen, Sanofi, Takeda, BMS, Roche, Pfizer, AstraZeneca, Novartis, Lilly. P. Espinosa Olarte: Financial Interests, Personal, Advisory Board: MSD, BMS, Johnson & Johnson; Financial Interests, Personal, Invited Speaker: MSD, Johnson & Johnson, AstraZeneca, Regeneron, Pfizer, BMS, Pierre Fabre, Roche; Non-Financial Interests, Personal, Principal Investigator, Clinical trial: Regeneron; Non-Financial Interests, Personal, Member: GECP, SEOM. M. Domine Gomez: Financial Interests, Personal, Advisory Board: AstraZeneca, Pfizer, ABBOT, Johnson & Johnson; Financial Interests, Personal, Invited Speaker: MSD oncology, Roche, Takeda. U. Dafni: Non-Financial Interests, Personal, Leadership Role, Member of the BoD & Manager of this not for profit organization providing statistical expertise and support in Clinical trials, primarily in cancer: Frontier Science Foundation - Hellas. S. Peters: Financial Interests, Institutional, Advisory Board: Vaccibody, Takeda, Seattle Genetics, Sanofi, Roche/Genentech, Regeneron, Phosplatin Therapeutics, PharmaMar, Pfizer, Novartis, Mirati, Merck Serono, MSD, Janssen, Incyte, Illumina, IQVIA, GSK, Gilead, Genzyme, Foundation Medicine, F-Star, Eli Lilly, Debiopharm, Daiichi Sankyo, Boehringer Ingelheim, Blueprint Medicines, Biocartis, Bio Invent, BeiGene, Bayer, BMS, AstraZeneca, Arcus, Amgen, AbbVie, iTheos, Novocure, Nykode, Nuvalent, Nuvation Bio; Financial Interests, Institutional, Invited Speaker: Takeda, Sanofi, Roche/Genentech, RTP, Pfizer, PRIME, PER, Novartis, Medscape, MSD, Imedex, Illumina, Fishawack, Eli Lilly, Ecancer, Boehringer Ingelheim, BMS, AstraZeneca, OncologyEducation, RMEI, Mirati; Financial Interests,